



- Demographic Characteristics
- Household Consumption and Poverty
- Health
- Education
- Housing
- Social Protection

Jamaica Survey of Living Conditions 2021



A Joint Publication of
The Planning Institute of Jamaica (PIOJ) and
The Statistical Institute of Jamaica (STATIN)

OVERVIEW »

INTRODUCTION

The 2021 Jamaica Survey of Living Conditions (JSLC) was fielded approximately one year after the first case of the novel Coronavirus Disease 2019 (COVID-19) was detected in Jamaica.¹ This pandemic had far-reaching effects on the social and economic welfare of households. Remedial actions taken to limit the spread of the many virulent strains of the virus included the wearing of masks and other protective gear, physical distancing, quarantines, stay-at-home orders, curfews and vaccination programmes. This period of adjustment and coping resulted in a shift in lifestyle, which affected the conduct of business, the provision of health care, education delivery and assimilation, access to social services, and conducting routine activities. Many social and economic interventions were mobilised across government, private sector, civil society and citizens in general to mitigate the negative impact of the shock to families, workers, businesses, students and other vulnerable groups. While many of these actions yielded positive results, the effects of the pandemic persist.

The JSLC continues to track the welfare of the population, including during and after periods of shock to ensure that ‘no one [is] left behind’. The 2021 data provide insight into the medium-term effects that the pandemic may or may not have had on the population, supported by comparisons with pre-COVID-19 conditions.

Using the Report

The 2021 report includes six chapters that present descriptive analysis on relevant socioeconomic data collected in June to October of that year.² The sample included 6 215 individuals from 2 187 households across the island. Detailed information on the methodology, survey design and other technical features of the survey—for example, estimating poverty and annualising the expenditure data—are included as appendices.

Due to the restrictions that were in effect and the need to limit the spread of the coronavirus among households and fieldworkers, no survey was fielded in 2020. As a result, the report presents comparisons with 2019 and, in some instances, over a five-year period with 2017. Additionally, the 2021 Survey was condensed to reduce contact time in the field and to examine only the issues considered most urgent and relevant to the social climate at the time.

To facilitate a shorter instrument, the following sections/indicators were suspended: early childhood development, screening tests, smoking, waiting time, perception of general health status, school attendance, distance travelled to school, and the school feeding programme. Questions were added to explore how students interacted with school due to the suspension of face-to-face classes. The survey also queried if a fear of COVID-19 influenced health-seeking behaviour, however, the numbers were too small for meaningful analysis. Additionally, the questions seeking to measure

1. COVID-19 was first confirmed in Jamaica in March 2020.

2. An estimated 97.9 per cent of the fieldwork was completed between June and September.



disability were changed to the Washington Group Short Set on Functioning in order to be consistent with international best practice.³

The data are analysed at the household and individual levels and where applicable disaggregated by region—the Greater Kingston Metropolitan Area (GKMA), Other Urban Centres (OUC), and Rural Areas—socio-economic quintile, age and sex. Standard Tables, which present current year data for each chapter, are located at the back of the report; time-series tables and charts are included in the chapters. Estimates in tables with coefficients of variation that exceed 20.0 per cent are italicised to encourage caution when analysing or reporting those estimates.

The JSLC is the collaborative effort of the Statistical

Institute of Jamaica, that collects the data, and the Planning Institute of Jamaica (PIOJ), that analyses the data and disseminates the findings. The JSLC datasets (1988–2021)⁴ are made available at the Derrick Gordon Data Bank, the University of the West Indies, Mona, and at the PIOJ for independent research. Researchers are encouraged to request the data and dig deeper beyond the findings included in this report to explore more fully the wealth of information contained in the JSLC.

EXECUTIVE SUMMARY

The Jamaican population continues to age as the proportion of children (0–14 years) in the population

3. See the Washington Group on disability statistics at <https://www.washingtongroup-disability.com/>

4. Datasets are available for each year except 2011 (due to the 2011 Population and Housing Census) and 2020. Note that there is a break in the series at 2017 due a change in the sampling design and weighting methodology.

continued its downward movement over time to 21.8 per cent. The working age population (15–64 years) is around its peak, 68.0 per cent, and the dependent elderly (65+ years) increased to 10.3 per cent. Consistent with a declining child population is a smaller average household size, which was 2.8 persons in 2021. Female-headed households continue to be characterised as larger and having more children and adult females than male-headed households. By socioeconomic status, compared with 2019, a larger share of households in the poorest quintile was categorised as medium- to large-sized households (by 3.1 percentage points to 56.9 per cent) and a larger share was also living in the GKMA (by 6.6 percentage points to 18.9 per cent).

Under normal conditions, the demographic characteristics of a population are not expected to change within a short period of time, therefore, with the exception of the changes seen by quintile, the structure and composition of the Jamaican household was similar to that observed in 2019. The relatively low numbers of deaths⁵ due to the coronavirus pandemic, therefore, did not result in any notable effect on the structure and composition of the population.

Nominal mean per capita consumption increased to \$430 516.00, by 4.7 per cent in real terms relative to 2019. This increase was not sufficient to prevent an upward movement in poverty and inequality. The prevalence of poverty increased to 16.7 per cent from 11.0 per cent in 2019. Rural Areas had the largest prevalence (22.1 per cent) and the largest increase (7.9 percentage points). The rate in OUC was 15.5 per cent (relatively unchanged) and in the GKMA 10.4 per cent (an increase of 5.7 percentage points). Inequality increased with the Gini coefficient at 0.3991 from 0.3671 in 2019. Congruous with a higher poverty rate is an increased allocation of household spending towards food. Food & non-alcoholic beverages accounted for a larger share of spending on consumption, an increase of 3.6 percentage points to 35.3 per cent compared with 2019. This translated to an average \$151 926.00

per person, a real increase of 12.7 per cent. Decreased spending was observed in five areas—transportation; clothing & footwear; education; recreation & culture; and restaurants & accommodation—usually associated with individuals spending more time at home. The changes observed in poverty and other socio-economic conditions suggest that the pandemic had major effects on the living conditions of the population.

Reported illness/injury during the four-week reference period was 4.8 per cent compared with 6.7 per cent in 2019. Of those who were ill/injured, there was an increase in the proportion that sought care (by 6.5 percentage points) and purchased medication (by 4.3 percentage points) to 76.6 per cent and 74.0 per cent, respectively. There was also an uptick in the proportion that accessed the public health care system. The majority of those who sought care went exclusively to a public facility (52.1 per cent), an increase from 46.6 per cent in 2019. In addition, of those who sought care, there was an increase of 8.4 percentage points in the proportion who went to a primary health care facility, to 67.4 per cent. Regarding chronic illnesses, the prevalence of reported non-communicable diseases (26.8 per cent) was similar to 2019, and remains a concern especially among persons 65+ years and older who registered a prevalence of 72.3 per cent. Health insurance coverage in the general population was 21.4 per cent.

Compared with 2019, school enrolment was relatively stable for children 6–16 years old, while changes were observed for individuals 3–5 years, 17–18 years and 19–24 years. Gross enrolment for those 3–5 years (93.0 per cent) and 17–18 years (62.3 per cent) fell by 6.7 and 4.3 percentage points, respectively. While for those 19–24 years, an increase of 3.8 percentage points to 20.1 per cent was observed. Specific to students enrolled at the early childhood, primary and secondary levels, when asked how they interacted with school during the last term of the academic year, 92.0 per cent reported that they participated in learning

5. Ministry of Health and Wellness Clinical Management Summary as at December 31, 2021. <https://www.moh.gov.jm/clinical-management-summary-for-friday-december-31-2021/>

activities, 83.8 per cent of whom did so via an online classroom. There was also a decline in the proportion of students that had all the required and supplemental textbooks to 67.2 per cent from 79.1 per cent in 2019, which may be partly attributed to the suspension of face-to-face classes. Overall, there was a decline in education-related spending for students at the primary and secondary levels.

For the out-of-school population, compared with 2019, the proportion that reported receiving skills training was relatively unchanged. However, there was improvement in the proportion that reported having certification. For persons 14+ years old, there was a 5.9 percentage-point increase in the proportion that indicated that they had some form of examination passes. At 15.9 percentage points relative to 2019, this increase was even higher for those of prime working age (25–54 years) as approximately a half of that cohort reported having some form of certification.

The Housing Quality Index increased to 78.8 per cent by 3.1 percentage points indicating an improvement in the quality of the housing stock relative to 2019. This increase was influenced by the proportion of households living in dwellings with Outer Walls of Concrete Block & Steel and those with Indoor Tap/Pipe as the Main Source of Drinking Water. As overall housing quality improved, the spread of households with secure tenure remained relatively stable. Approximately 30.7 per cent of households reported that they owned the dwelling, the land and had a registered title for the property compared with

29.1 per cent in 2019. Positive changes were observed in the area of ownership and use of information and communication technologies mainly due to restrictions during the COVID-19 pandemic—48.8 per cent of households reported having both internet access and a working computer, an 11.8 percentage point increase compared with 2019. Regarding spending on household-related items, increases were reported for water, telephone, electricity, mortgage and property tax.

Approximately 42.6 per cent of households felt that their economic situation was worse off in 2021 than during the height of the pandemic in 2020, with an additional 42.8 per cent feeling that their situation had not changed. Regarding food security, compared with 2019, a smaller proportion of households (52.3 per cent) felt that they had enough food to eat on a daily basis, down by 9.7 percentage points. Registration on various social security programmes remained low among the eligible population groups—National Insurance Scheme (NIS), 48.7 per cent, National Health Fund (NHF), 24.0 per cent, and the Jamaica Drug for the Elderly Programme (JADEP), 17.4 per cent. There was no change in the proportion of households (32.5 per cent) that applied to the Programme of Advancement through Health and Education (PATH) relative to 2019, however, there was an increase in the proportion of applicant households that received a benefit, to 63.5 per cent from 58.8 per cent. Only 32.5 per cent of the elderly (60+ years) population were receiving a pension.

“APPROXIMATELY 42.6 PER CENT OF HOUSEHOLDS FELT THAT THEIR ECONOMIC SITUATION WAS WORSE OFF IN 2021 THAN DURING THE HEIGHT OF THE PANDEMIC IN 2020, WITH AN ADDITIONAL 42.8 PER CENT FEELING THAT THEIR SITUATION HAD NOT CHANGED.”

CHAPTER SUMMARIES

Demographic Characteristics

Jamaica is at an advanced stage of the demographic transition with the age distribution of the population showing the proportion of children (0–14 years) at 21.8 per cent, persons of working age (15–64 years), 68.0 per cent, and the dependent elderly (65+ years), 10.3 per cent. This resulted in an age dependency ratio (ADR) of 47.2 dependents per 100 persons of working age compared with 48.1 in 2019. The highest level of dependency was found in female-headed households in Rural Areas (ADR, 56.0) and lowest in male-headed households in the GKMA (ADR, 35.9). As expected, the survey showed no significant shifts in the age structure of the population between 2021 and 2019.

The average household size was 2.8, comprised of 1.0 adult males, 1.2 adult females and 0.6 children. A look at the distribution of households by size showed that nationally, seven in every 10 households (70.2 per cent) had 1–3 members, while 25.5 per cent had 4–6 members and 4.3 per cent had 7 or more. By sex of head, female-headed households were, on average, larger (3.1) with fewer adult males (0.7), and more adult females (1.6) and children (0.8). The corresponding figures for male-headed households were 2.6, 1.3, 0.8 and 0.5, respectively. One-third (32.6 per cent) of female-headed households had four or more members compared with 27.5 per cent of male-headed households. The proportion of female-headed households with at least one child present was 47.7 per cent relative to 29.3 per cent of male-headed households.

The majority of households in the poorest quintile consistently lives in Rural Areas, 60.9 per cent in 2021. There was an increase in the share that lived in the GKMA to 18.9 per cent from 12.3 per cent in 2019. Quintile 1 also had the largest average household size of 4.2, increasing from 4.0 in 2019. Approximately 56.9 per cent of households in this quintile had four or more members up from 53.8 per cent in 2019. The mean household size in Quintile 5 was relatively unchanged.

Household Consumption and Poverty

On average, individuals spent more in 2021 (\$430 516.00) than in 2019 (\$370 853.00); an increase of 4.7 per cent in real terms. By region, nominal mean per capita consumption expenditure was highest in the GKMA at \$554 623.00, followed by OUC \$401 407.00 and Rural Areas \$349 333.00. Individuals in male-headed households spent an average \$479 746.00 compared with \$380 412.00 for female-headed households. Compared with 2019, these reflect increases in real dollars by 7.1 per cent and 1.7 per cent, respectively, indicating a widened disparity.

Four commodity groups—food & non-alcoholic beverages; housing, water, electricity, gas and other fuels; transportation; and restaurants & accommodation services—accounted for 75.5 per cent of household expenditure. Food & non-alcoholic beverages continued to account for the largest share of consumption expenditure with an increase to 35.3 per cent of consumption from 31.7 per cent in 2019. At \$151 926.00, average expenditure on food & non-alcoholic beverages increased by 12.7 per cent in real terms. There was also an increase for housing, water, electricity, gas and other fuels to \$47 664.00 by 26.7 per cent. Declines were observed for transportation (\$51 809.00) and restaurants & accommodation services (\$52 277.00), which declined by 7.4 per cent and 16.0 per cent, respectively. These declines are consistent with persons restricting their activities outside of the home.

In relation to remittances, 50.0 per cent of households reported receiving remittances from overseas, 5.9 percentage points lower than in 2019. Declines were also observed by region, quintile and sex of household head, except for OUC and Quintile 4, which were relatively unchanged. The largest decline by region and quintile were seen in the GKMA by 11.3 percentage points (to 52.3 per cent) and in Quintile 2 by 8.5 percentage points (to 47.1 per cent); male-headed and female-headed households declined by 5.3 and 6.6 percentage points to 47.8 per cent and 52.8 per cent, respectively.

The Gini coefficient confirmed that inequality

in the country had increased. The Gini coefficient was 0.3991, an increase from 0.3671 in 2019. In real terms, individuals in the poorest quintile reduced consumption by 2.6 per cent, while those in Quintile 5 increased it by 9.4 per cent relative to 2019. Mean per capita consumption expenditure of the wealthiest quintile (\$951 026.00) exceeded that of the poorest quintile (\$123 662.00) 7.7 times compared with 6.8 times in 2019. At the level of deciles, the gap between the poorest and wealthiest deciles was also wider in 2021 at 12.5 times relative to 10.9 times in 2019.

The prevalence of poverty was 16.7 per cent in 2021 indicating that this portion of the population was consuming below the 2021 poverty line of \$223 162.00. Relative to 2019, this was a 5.7 percentage point increase in the prevalence of poverty and a reflection of the continued negative effect of COVID-19 on individuals. Regionally, the prevalence of poverty was highest in Rural Areas at 22.1 per cent followed by OUC at 15.5 per cent and the GKMA with 10.4 per cent. Compared with 2019, increases were observed in Rural Areas and the GKMA by 7.9 and 5.7 percentage points, respectively. The rate in OUC was statistically similar to that recorded in 2019. While the poverty rates increased for all age groups, children and youth continued to have higher rates of poverty, and the margin of increase was larger among children than among the elderly. For children 0–17 years, the prevalence of poverty was 22.1 per cent and for the elderly, 60+ years, 13.9 per cent, registering increases of 8.8 percentage points and 4.5 percentage points, respectively.

The 2021 data were consistent with the historical trend of a higher poverty rate for individuals in female-headed households than in male-headed households—19.4 per cent and 14.0 per cent, respectively. Households headed by females also had a larger increase in poverty compared with 2019. The prevalence of poverty increased in female-headed households by 8.5 percentage points and 2.9 percentage points in male-headed households. In 2019, there was no statistical difference between the two household types. Additionally, in 2021, for individuals in households with children, the prevalence of poverty

was almost double that of individuals in households without children irrespective of the sex of the head. In female-headed households with children the poverty prevalence was 22.9 per cent and in those without children, 11.8 per cent. In male-headed households with children, the poverty prevalence was 18.6 per cent and in those without, 9.0 per cent. For individuals in these four household types, relative to 2019, those in female-headed households with children had the largest increase in poverty of 10.8 percentage points, while for those in male-headed households with no children it was relatively unchanged.

On average, poor individuals had a mean consumption expenditure that was 53.1 per cent of the poverty line, down from 64.8 per cent in 2019. The depth of poverty (poverty gap) was 4.4 per cent and the severity of poverty (squared poverty gap) was 0.018 compared with the respective 2019 figures of 3.1 per cent and 0.012.

Health

Approximately 4.8 per cent of individuals reported being ill or injured in 2021 during the four-week reference period preceding the survey, compared with 6.7 per cent in 2019. While there was no noticeable distinction by region, quintile or sex, a larger proportion of the elderly (60+ years) reported illness/injury, 9.1 per cent, relative to those in the prime working age (25–59 years), 4.9 per cent; and younger (0–24 years), 3.1 per cent; children (0–17 years) 5.8 per cent. The 2021 estimates for each age group were within 2.0 percentage points below those for 2019. On average, persons with illness/injury reported being ill/injured for almost 9.0 days and impaired for almost 5.0 (almost 8.0 and 4.0 days, respectively in 2019). Of those who were ill/injured, three-quarters (76.6 per cent) sought medical care and 74.0 per cent bought medication, compared with 70.1 per cent and 69.7 per cent, respectively, in 2019.

Persons sought health care exclusively at a public or a private health facility in 2021; unlike previous years, the survey did not identify anyone who visited both.

The majority of individuals who were ill or injured (52.1 per cent) sought care exclusively at a public health facility (46.6 per cent in 2019). This was also true for persons who lived in Rural Areas (63.3 per cent), in Quintiles 1 to 3 (approximately 61.0 per cent each), both males (52.5 per cent) and females (51.9 per cent), persons 0–24 years (59.2 per cent) and the elderly (54.3 per cent). The 2021 data was similar to 2019 in that the majority of persons in the lower quintiles, children and the elderly accessed public health services. There was an increase in the proportion of persons who sought care who accessed primary care services, to 67.4 per cent by 8.4 percentage points relative to 2019. In 2021, among the ill/injured who did not seek care, the main reason given was that the individual did not feel sick enough (56.9 per cent). With regard to medication, the majority of persons accessed private pharmacies exclusively at the national level (77.9 per cent) and across region, quintile, sex and age group, similar to 2019.

On average, each individual who visited a private health facility during the reference period spent \$5 223.00, and \$6 963.00 at private pharmacies, compared with \$2 803.00 and \$5 262.00, respectively in 2019. Expenditure was not analysed for public health care facilities due to too few responses.

Approximately one in every four individuals (26.8 per cent) reported being diagnosed with at least one non-communicable disease (NCD), relatively unchanged from 2019. The prevalence remains consistently higher among females (31.1 per cent) and older persons (72.3 per cent of persons 65+ years old). The three most common NCDs were hypertension (15.0 per cent), asthma (7.6 per cent) and diabetes (5.9 per cent).

Health insurance coverage among the population was 21.4 per cent. This was, however, a 3.2 percentage-point increase compared with 2019. Coverage in Rural Areas (12.2 per cent) and Quintiles 1 and 2 (6.5 per cent each) was significantly behind other regions and quintiles. Although the prevalence of chronic diseases was high among the dependent elderly population, health insurance coverage was disproportionately low at 24.7 per cent.

Education

The gross enrolment rate for the overall school-aged population remained stable in 2021 compared with pre-COVID-19 enrolment in 2019. By age, this was also true for persons 6–16 years old; however, declines were observed for persons 3–5 years old by 6.7 percentage points to 93.0 per cent, and for those 17–18 years by 4.3 percentage points to 62.3 per cent. There was an increase in gross enrolment for individuals 19–24 years (post-secondary/tertiary level) by 3.8 percentage points to 20.1 per cent. By region, the decline in enrolment for those 3–5 years was more obvious in the GKMA (by 10.3 percentage points) and OUC (by 7.6 percentage points) than in Rural Areas (by 3.8 percentage points). Conversely, the overall decline for those 17–18 years was due to a 9.7 percentage point decline in Rural Areas, as gross enrolment in the GKMA and OUC was relatively unchanged. At the post-secondary/tertiary level, an increase in enrolment was observed only for those who lived in the GKMA (by 6.4 percentage points).

A look at gross enrolment by sex showed that the disparity between male and female enrolment was within 2.5 percentage points at all levels with two exceptions. For persons 17–18 years, gross enrolment among males was 53.1 per cent and females 74.4 per cent. Compared with 2019, male enrolment for this age declined by 17.1 percentage points and female enrolment increased by 12.4 percentage points resulting in an enrolment gap (21.3 percentage points) that more than doubled the gap in 2019 (8.2 percentage points). For persons 19–24 years, the disparity was 7.0 percentage points, with enrolment for males 16.8 per cent and females 23.7 per cent (13.7 per cent and 19.2 per cent, respectively in 2019).

For net enrolment, relative to 2019, a difference was observed at the national level for two age groups, individuals 3–5 years (92.1 per cent), which declined by 4.2 percentage points, and those 19–24 years (18.7 per cent), which increased by 3.8 percentage points. Disaggregation by region for both age groups showed similar patterns to the gross enrolment rates, in that, for persons 3–5 years, the decline in net enrolment

was notably larger in urban areas than rural and for those 19–24 years, the increase in net enrolment was largest in the GKMA. The change was similar for males and females in both age groups relative to 2019. For individuals 17–18 years, while there was no change in net enrolment at the national level (52.7 per cent), by region, enrolment for those living in the GKMA increased by 3.5 percentage points, declined in OUC by 6.5 percentage points and registered no change in Rural Areas. In the 17–18 age group, the gap in net enrolment widened, female enrolment increased by 3.9 percentage points while that of males decreased by 3.4 percentage points.

In response to the reality that students were required to engage in remote learning during the pandemic, the survey examined the level and nature of student's engagement with school during May to July 2021 (the school term closest to the fielding of the survey). Of students enrolled at the early childhood through to the secondary level (3–18 years), 92.0 per cent reported participating in learning activities provided by the school, 90.7 per cent of males and 93.4 per cent of females; 96.1 per cent from Quintile 5 and 90.0 per cent from Quintile 1. Of those who participated, 83.8 per cent interacted with the teacher via online classroom and 9.9 per cent did so face-to-face. Of those who did not engage with school, the three main reasons given for non-participation were no access to the internet (36.1 per cent), school was closed (28.8 per cent) and they could not afford to (11.0 per cent).

An examination of the proportion of students that had textbooks showed that 67.2 per cent had all the required and supplemental textbooks, a decline of 11.9 percentage points relative to 2019. A decline was also seen by region, sex and quintile and was largest in Rural Areas by 13.6 percentage points to 61.0 per cent, among males by 14.1 percentage points to 65.0 per cent and in Quintile 5 by 17.9 percentage points to 77.3 per cent. Quintile 1 declined by 11.0 percentage points to 51.5 per cent. Overall, the proportion of students with some of the required and supplemental books was 6.5 per cent, 8.1 per cent had only the required texts, while 11.1 per cent had none. Some 21.9 per cent of the students in

the poorest quintile had no textbooks.

There was a general decline in household expenditure on education for students at the primary and secondary levels compared with two years prior whether or not the school was public or private. For public primary schools, the highest average expenditure was on books (\$8 547.00). Declines were recorded for all categories with the largest recorded for transportation to \$2 880.00 by 88.9 per cent in real terms. In private primary-level schools, transportation also registered the highest reduction at 80.0 per cent. The highest average expenditure in these private schools was on tuition (\$127 954.00), which increased by 7.0 per cent in real terms, and auxiliary and other contributions (\$123 071.00), which was relatively similar to 2019. Spending on education for students attending public secondary schools decreased for all categories, except for examination fees, which was relatively unchanged. Consistent with the primary levels, relative to 2019, expenditure on transportation (\$9 131.00) registered the largest decline, by 78.2 per cent in real terms. The sample was too small to adequately assess expenditure on education for students who attended private secondary-level schools.

There was positive movement regarding certification among the 14+ year old population, not enrolled in any school/education institution, over the two years since 2019. This was supported by a 5.9 percentage-point increase to 45.0 per cent in the proportion of this group that indicated that they had some form of examination passes. As expected, the proportions with certification decreased with age, however, improvement was most marked among the 60+ years age group with 11.7 percentage points to 24.4 per cent. Those 35–49 years followed closely behind as they registered a 10.5 percentage point increase to 47.8 per cent. Compared with 2019, the level of increase among the other age groups ranged from 2.3 percentage points among those 14–19 years to 4.8 percentage points among those 50–59 years. There were no significant patterns of increase by region, quintile or sex. For the prime working age population (25–54 years), approximately a half (50.7 per cent) had some form of certification, reflecting an

increase of 15.9 percentage points compared with 2019. Regarding skills training, of those 14+ years and out of school, 21.9 per cent had at some point in their lives received skills training; the 2019 figure was 21.0 per cent.

Housing

There was an observed improvement in the quality of the housing stock since 2019. This was supported by a 3.1 percentage point increase in the Housing Quality Index to 78.8 per cent. Of the six component indicators, two showed an improvement, the proportion of households living in dwellings with Outer Walls of Concrete Block & Steel (78.7 per cent) and those with Indoor Tap/Pipe as the Main Source of Drinking Water (58.4 per cent) by 5.7 percentage points and 11.9 percentage points, respectively. The remaining four indicators—Electricity for Lighting (96.0 per cent), Exclusive Use of Kitchen (94.1 per cent), Exclusive Use of Water Closet (78.9 per cent) and Number of Persons per Habitable Room (66.5 per cent)—were relatively unchanged. The improvement in the HQI at the national level was influenced by increases in the GKMA by 5.7 percentage points and OUC by 6.1 percentage points to 86.4 per cent and 82.6 per cent, respectively. Rural Areas have consistently lagged behind the other regions, and with an HQI of 71.6 per cent, showed no change relative to 2019.

Three in every 10 households (30.7 per cent) had registered full ownership, that is, they reported ownership of the dwelling, ownership of the land on which they live, as well as having a registered title for the property. The range across regions spanned 6.3 percentage points, 34.4 per cent in the GKMA to 28.1 per cent in Rural Areas. When each indicator is taken individually, at the national level, 55.6 per cent of households owned their dwelling, 43.9 per cent owned the land on which they live, and 69.0 per cent of those that owned had a registered title.

The proportion of households that used an improved drinking water source remained relatively similar—75.2

per cent relative to 76.6 per cent in 2019. However, there was a widening of the gap between the urban and rural areas. The GKMA remained constant at a high of 99.3 per cent, and OUC increased by 4.5 percentage points to 91.7 per cent, while Rural Areas decreased 5.3 percentage points to 50.7 per cent of households. Nationally, there was an 11.9 percentage point increase in the proportion of households that used an indoor pipe as their main source to 58.4 per cent and a corresponding 11.3 percentage point decline in the proportion that used bottled water to 1.7 per cent. This shift may have been due to the increased number of individuals who were engaged in work and school from home, which limited their movement to purchase water. Of the households with piped water, almost 60.0 per cent had at least one water lock-off during the 30-day reference period before the survey. The average number of lock-offs⁶ was 7.7. Households stored water mainly in plastic tanks (44.3 per cent) and drums (36.7 per cent), and stored water lasted an average 12.5 days.

The majority of households (56.6 per cent) disposed of their garbage using a regular public collection system, while for 12.5 per cent the collection was also public but irregular, 25.1 per cent burned their garbage and the remainder used a private facility or dumped/buried their garbage. Nationally, compared with 2019, there was a positive shift in the proportion of households (by 2.8 percentage points) that used formal methods. This resulted from increases in the GKMA and OUC by 4.3 and 5.7 percentage points, respectively. In Rural Areas no positive movements were observed; the pervasiveness of informal methods, especially burning (44.8 per cent), remains a concern.

On average, households generally spent more on household expenses overall than in 2019. There were increases in real terms for mean monthly water payments by 50.8 per cent (\$4 800.00), telephone by 36.4 per cent (\$6 593.00), property tax by 19.6 per cent (\$1 057.00), electricity by 3.7 per cent (\$8 238.00) and mortgage by 2.6 per cent (\$30 884.00). However,

6. A 'lock-off' is defined as any disruption of supply regardless of the length of time of disruption. Only one per calendar day is counted.

per cent had enough food sometimes and 15.8 per cent did not. These figures indicate a decline in perceived food security relative to the pre-pandemic conditions in 2019. Nationally, there was a 9.7 percentage point drop in the proportion of households that reported having sufficient food regularly. This was consistent within each region, quintile and in both male- and female-headed households. Rural Areas (13.3 percentage points) and OUC (12.6 percentage points) had more notable declines than the GKMA (3.1 percentage points). While both male- and female-headed households registered a decline in the proportion that had sufficient food regularly, the decline in male-headed households was markedly larger—by 13.2 percentage points relative to 5.3 percentage points in female-headed households. Among households headed by an elderly person (60+ years old), 53.1 per cent reported having enough food regularly, a decline of 4.3 percentage points relative to 2019. The survey also examined how households compared their economic situation in 2021 with the situation in 2020; some 13.1 per cent said their situation was better off, 42.8 per cent, equal/same and 42.6 per cent felt worse off.

In relation to pensions among the elderly population, 60 years and older, 32.5 per cent were in receipt of some form of pension whether it was from the National Insurance Scheme (24.8 per cent), related to their occupation (12.3 per cent), a private fund (5.1 per cent) or other facility (1.3 per cent). Coverage among the elderly in Rural Areas continue to lag behind urban areas by approximately one-half, and for those in the poorest quintile, one-third that of Quintile 5.

Coverage under select social security programmes remained low relative to the size of the eligible population. For the National Insurance Scheme (NIS), 48.7 per cent of individuals 18 years and older reported being registered, and of those registered, 82.9 per cent had contributed. For programmes that provide subsidy for medication, the National Health Fund (NHF) and the Jamaica Drug for the Elderly Programme (JADEP), respectively, 24.0 per cent of the population and 17.4

per cent of the elderly population were registered. These figures are lower than the prevalence of chronic diseases in the population and elderly population, respectively.

CONCLUSION

The positive movements in the 2021 macroeconomic indicators relative to 2020 verified that progress had been made towards recovering from the global health crisis.⁷ This progress would have tempered the negative short- to long-term impacts for households and the country as a whole. However, with the depth and reach of the pandemic's effect, the improvements that had been made were insufficient or inadequately distributed to restore all the gains that had been made prior to COVID-19. A year into the pandemic, the impact on the socioeconomic welfare of households was most marked in the areas of household expenditure, food security, education and ICT. Households on average allocated a larger portion of funds to food and other household-related items and amenities to assist them in adjusting to the increased need for remote work and school over the period. In addition, overall, there was an increase in household expenditure relative to 2019, but it was due to an increase among the wealthiest segment of the population as the poorest group spent less, contributing to the increase in the prevalence of poverty. The largest increases in poverty were among residents in rural areas and female-headed households, especially those with children.

A segment of the population demonstrated some level of resilience during the pandemic as approximately 4 in every 10 households expressed that they felt better off financially during 2020, when the pandemic had the most potent domestic impact, than they were doing in 2021, which may reflect a depletion of savings and other reserves. Another 4 of the 10 households reported being able to hold strain during the second year. Female-headed households and households headed by elderly members also showed

7. Planning Institute of Jamaica, *Economic & Social Survey Jamaica 2021* (Kingston: PIOJ, 2022).

greater resilience as seen regarding perceptions of their ability to provide sufficient food for their families, even though their spending capacity was generally less than before the pandemic. The data showed no increase in applications to the PATH programme, which may have been due to the breadth of support given to vulnerable families through the network of government, private sector, civil society and community effort during the height of the pandemic. Notwithstanding the wide scope of supportive measures implemented over the period, the increase in poverty and inequality, and the reduction in food security indicated that additional and sustained support is needed.

Heightened public education on proper sanitation and hygiene practices, provision of protective gear and restricted movements may have contributed to not having an increase in reported illness/injury in 2021 considering the highly contagious nature of the virus. The preventative measures may have protected persons against the spread of other viruses, injuries and other illnesses, not just COVID-19, which proved advantageous in mitigating the added burden created by the pandemic on the health care system. Of those who were ill/injured, larger proportions accessed care, specifically public and primary health care, and increased expenditure for health care relative to 2019. This would have added to the financial stress within those households. Ensuring that all families are able to adequately respond to health shocks while concurrently strengthening the capacity within the system is imperative, especially in the context of low health insurance coverage, low registration to the

NHF and JADEP, and continuing high levels of non-communicable diseases, particularly among the elderly.

Opportunities were created for expanded computer literacy and interface with information and communication technologies. This was evident in the proportion of students that engaged with schools remotely and the overall sharp increase in the ownership of working computers by households. The huge gap in computer ownership between the wealthiest and poorest segment of the population was significantly reduced compared with 2019. Further evidence was seen in the broader diversification of payment modalities for beneficiaries to PATH to include increased use of electronic payment options through money transfer and remittance agencies. There were also increases in the proportion of persons over the age of 35 years that had some form of certification. This may have been due to an increased number of courses being offered through mixed methods, affording individuals within the prime working age and older greater levels of flexibility to further their education while working.

Jamaica is not insulated from the impacts of global shocks and internal challenges that threaten advancements on the journey towards developed country status by 2030. The country's demographic profile indicates an ideal position to maximize the benefits from having a large working age population to strengthen human capital and livelihood resilience, accelerate rural development, and reduce vulnerability across all segments of the population.

“THE COUNTRY’S DEMOGRAPHIC PROFILE
INDICATES AN IDEAL POSITION TO MAXIMIZE THE
BENEFITS FROM HAVING A LARGE WORKING AGE
POPULATION TO STRENGTHEN HUMAN CAPITAL
AND LIVELIHOOD RESILIENCE . . .”